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CASE #	CASE NAME	HEARING NAME
CVME2511189	MALONE vs TEMECULA VALLEY HOSPITAL, INC.	MOTION TO COMPEL FURTHER RESPONSES TO REQUESTS FOR PRODUCTION OF DOCUMENTS, SET TWO

Tentative Ruling:

The meet-and-confer requirement has been met.

RFP No. 60 (clinical audits concerning patient falls that the corporate parent entity UHS provided to Defendant from 9/24/23 through 12/1/24)

Decedent was admitted to the hospital on 9/25/24. (Complaint, ¶ 5.) The request covers a 14-month window before and after Decedent's fall. Good cause is shown. The requested audits are probative of whether Defendant had knowledge of recurring falls in its facility and whether it deliberately disregarded them before and after Decedent's injury. In *Lopez v. Watchtower Bible & Tract Society of New York, Inc.* (2016) 246 Cal.App.4th 566, the court held that a defendant's post-incident reports are discoverable to show ratification and reprehensibility of that defendant's actions to support punitive damages. (*Lopez v. Watchtower Bible & Tract Society of New York, Inc.*, *supra*, 246 Cal.App.4th at p. 592.) "One relevant factor in this analysis is the extent to which the defendant's alleged wrongful conduct involved repeated actions, including conduct occurring after the incident in question." (*Ibid.*) A 14-month timeframe is not unreasonable. The requested audits are relevant to show recklessness.

Evidence Code section 1157

Defendant argues the audits are protected under Evidence Code section 1157. Section 1157 protects "the records of organized committees of medical, medical-dental, podiatric, registered dietitian, ... prehospital emergency medical care person or personnel, or veterinary staffs, or of a peer review body." (Evid. Code, § 1157(a).) It also provides that the proceedings and records of hospital staff committees "having the responsibility of evaluation and improvement of the quality of care rendered" are not subject to discovery. Peer review bodies described in Bus. & Prof. Code §805 are also protected. Moreover, persons who attended such meetings cannot be compelled to testify as to what transpired. (Evid Code §1157(b); *University of Southern California v. Superior Court* (1996) 45 Cal.App.4th 1283, 1290.) Evid. Code §1157 creates only a privilege against discovery from medical staff committees; it does not create a bar against introduction of evidence obtained from other sources. (*Alexander v. Superior Court* (1993) 5 Cal.4th 1218, 1224.)

"Section 1157 'applies *only* to records of and proceedings before medical investigative committees.' [Citation.] Information developed or obtained by hospital administrators or others which does not derive from an investigation into the quality of care or the evaluation thereof by a medical staff committee, and which does not disclose the

investigative and evaluative activities of such a committee, is not rendered immune from discovery under section 1157 merely because it is later placed in the possession of a medical staff committee or made known to committee members; and this may be so even if the information in question may be relevant in a general way to the investigative and evaluative functions of the committee.” (*Santa Rosa Memorial Hospital v. Superior Court* (1985) 174 Cal.App.3d 711, 724 [italics in original].)

To determine whether the privilege applies, the court in *Matchett v. Superior Court* (1974) 40 Cal.App.3d 623, 630-631 examined the functions of each relevant staff committee to determine whether it fit the statutory description: of “having the responsibility of evaluation and improvement of the quality of care rendered in the hospital”, only to conclude that the Credentials Committee, Records Committee, Tissue Committee, and Executive Committee all fell within the scope of the privilege. In *Santa Rosa Memorial Hospital*, by contrast, the court noted that administrative activities which, “while related to, are independent of the investigative and evaluative activities of medical staff committees” are not protected by §1157 (*Santa Rosa Memorial Hospital, supra*, 174 Cal.App.3d at p. 726).

Hospitals have a dual structure. First, an administrative governing body (often comprised of persons other than health care professionals) takes ultimate responsibility for the quality and performance of the hospital. Second, an “organized medical staff” entity (composed of health care professionals) has responsibility for providing medical services, and is “responsible to the governing body for the adequacy and quality of the medical care rendered to patients in the hospital.” [Citations.]

The medical staff entity is required to perform various functions (e.g., “executive review, credentialing, ... utilization review, infection control”) through one or more committees. [Citation.] State regulations require that medical staff committees have “formal procedures for the evaluation of staff applications and credentials, appointments, reappointments, [and] assignment of clinical privileges ...” [Citation], and that applicants and reapplicants for hospital privileges must “demonstrate their ability to perform surgical and/or other procedures competently and to the satisfaction of an appropriate committee or committees of the staff, at the time of original application for appointment to the staff and at least every two years thereafter.” [Citation.]

(*Alexander v. Superior Court* (1993) 5 Cal.4th 1218, 1224 (disapproved of on other grounds as stated in *Hassan v. Mercy American River Hospital* (2003) 31 Cal.4th 709)); see also *Willits v. Superior Court* (1993) 20 Cal.App.4th 90, 100.)

Willits, supra, was an action by a nurse who was infected with HIV following a needle stick injury. The plaintiff sought discovery of various records from the hospital, which asserted the §1157 privilege. The court held that the §1157 privilege could apply to actions by hospital employees but cautioned against reading that holding too broadly:

“we fully endorse the distinction that Matchett, Santa Rosa Memorial Hospital, and other cases have consistently drawn between administrative functions/records and committee functions/records.” (*Willits, supra*, 20 Cal.App.4th at p.103.)

The precise functions of the clinical audits are not fully described by the parties, as Defendant Temecula Valley Hospital failed to provide a privilege log or explain the generation and routing of the documents in its response to RFP no. 60; the opposing papers provided to the court fail to identify any specific committee, meeting, or author involved in the audits. “[A] hospital cannot receive the benefit of section 1157 if it refuses to bear the associated burden of demonstrating why the information claimed to be immune should be deemed a record or proceeding of a medical staff committee.” (*Santa Rosa Memorial Hospital, supra*, 174 Cal.App.3d at p. 727.) Such a showing seems particularly appropriate here as the clinical audits sought in RFP no. 60 were generated by co-defendant UHS of Delaware, Inc. (“UHS”), which both defendants affirmatively plead is a separate corporate entity. Furthermore, the declaration of defense counsel Saira M. Mejia states Defendant’s risk management and clinical audit materials are prepared for “claims handling and litigation-related review” and that “[t]he reports do not serve any other purpose.” (Mejia Decl., ¶ 6). There is simply no evidence before the court to conclude that the audits that UHS provided were records or proceedings of an internal committee at the hospital with the “responsibility of evaluation and improvement of the quality of care rendered in the hospital....” (Evid. Code §1157(a).) Accordingly, the audits fall outside the protection of §1157.

Patient Safety and Quality Improvement Act of 2005

Defendant also argues the requested audits are privileged “patient safety work product” under the federal Patient Safety and Quality Improvement Act of 2005 (“the Act”). (42 U.S.C. § 299b-21 et seq.) Under the Act, notwithstanding other state and federal law, patient safety work product shall not be subject to discovery in state civil matters against a provider or admitted as evidence in a civil proceeding. (42 U.S.C. § 299b-22(a).) Patient safety work product means data, reports, memoranda, records, analyses, or statements that are assembled by a provider for reporting to a patient safety organization and are reported to a patient safety organization or developed by a patient safety organization for conducting patient safety activities. (42 U.S.C. § 299b-21(7)(A). This information does not include “a patient’s medical record, billing and discharge information, or any other original patient or provider record.” (42 U.S.C. § 299b-21(7)(B)(i).) This provision should also not be construed to limit “the discovery of or admissibility of information described in this subparagraph in a criminal, civil, or administrative proceeding.” (42 U.S.C. § 299b-21(7)(B)(ii).)

Defendant argues the audits are protected patient safety work product because its corporate parent UHS is a federally registered Patient Safety Organization. (Mejia Decl. ¶ 7.) However, 42 C.F.R. § 3.20 expressly excluded from the definition of patient safety work product information that is “collected, maintained, or developed separately, or exists separately, from a patient safety evaluation system.” (42 C.F.R. § 3.20(2)(i).)

Here, none of the audits sought in RFP no. 60 involve a patient safety evaluation system. Instead, the request is limited to “clinical audits that mentioned or discussed patient falls.” (Separate Statement, p. 2.) Defendant’s own counsel also acknowledges these audits are prepared and maintained for “claims handling and litigation-related review” and “do not serve any other purpose.” (Mejia Decl., ¶ 6). This places the requested audits outside of the protection of patient safety work product. Defendant has also presented no evidence showing that the audits involve a patient safety evaluation system. The website printout attached to the Mejia Declaration merely establishes that “UHS Acute Care PSO” is a listed Patient Safety Organization. It does not show these specific fall audits were created in connection with a patient safety evaluation system. (Mejia Decl., Ex. A). The audits do not fall under the protection of the Act.

Attorney-Client Privilege and Attorney Work-Product Doctrine

Defendant further asserts attorney-client and work-product privileges. The party asserting the privilege must present facts supporting a prima facie claim of privilege; only then does the opposing party have the burden of showing that the privilege does not apply, an exception applies, or there was a waiver. (*Oxy Resources California, LLC v. Superior Court* (2004) 115 Cal.App.4th 874, 894.) Defendant provides only generic statements in its opposing memorandum and a declaration from outside counsel. Further, Defendant did not provide a privilege log. (Code Civ. Proc., § 2031.210(c)(1).) Accordingly, Defendant failed to justify these privileges.

Sanctions

Sanctions are mandatory against a party who unsuccessfully makes or opposes a motion to compel unless the court finds that the party acted with “substantial justification” or that other circumstances make the imposition of sanctions unjust. (Code Civ. Proc., § 2031.310(h).) Defendant asserted broad privilege objections without a privilege log or specific factual support. Plaintiff’s request for \$825 in sanctions is reasonable. **The court awards \$825 in sanctions.**

The motion to compel further production is granted. Sanctions are awarded in favor of Plaintiff and against Defendant Temecula Valley Hospital, Inc. and its counsel of record in the amount of \$825, to be payable within 30 days of the court’s order. Defendant Temecula Valley Hospital, Inc. is further ordered to provide a further response to RFP, Set Two, no. 60 within 30 days of the court’s order.

Moving party is ordered to submit by no later than July 13, 2026, by 5:00 p.m., a proposed order consistent with this ruling.